



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : إيمان محمد محمود سليم
المحافظة : القليوبية
الحى / المدينة : الخصوص
رقم البطاقة : 27605010101744 رقم التليفون : 01122886191
العمر (تاريخ الميلاد) : 01-05-1976 المهنة : ربه منزل
العنوان : سعيد
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/رمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبى للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
.....

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبى المتخصص

Name: **أيمن محمد محمود علي**
National ID: **37605010101744**



تقرير طبي

المريض

تحية طبية وبعد،،

بشأن تقرير طبي للمريض

إهداء للطلب المقدم من

رقم طبي:

رقم:

تشرف بأن أوضح أيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الشغل :
- - - - - appropriate

وبعد الشكر والموصات والمتابعة وجدته

المرحلة الحالي من يوم غرضه والشيء و يحتاج الى Supportive and palliative Treatment

توقيع :

أحمد محمد علي

التاريخ :

05-07-2026

الطبيب المعالج :
الأستاذ الدكتور



Patient Name : الأسبلاء / ايمان محمد محمود سليم	Request No. : 13005623
Patient No. : 27605010101744	Request Date : 09/04/2025
Path No. : 1246/25	Verification Date : 29/04/2025
Sex : Female	Financial Category : اورام ناز السلام - مياثره
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Right wide local excision and axillary clearance specimens, collected at 9/4/2025.

Gross Examination:

Five specimens were received:

-Axillary clearance specimen : Two Fibrofatty tissue pieces measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.

- Extra medial margin: Fibrofatty tissue piece measured 3x2.5 cm

- Extra deep margin: Fibrofatty tissue piece measured 2.5x2 cm

-Right wide local excision specimen measured: 10x9x4 cm with covering skin ellipse measured 9x3.5 cm.

- Tumor extent

o Number: Two (wired) ill defined rubbery greyish white areas.

o Size: 1.5x1x1 cm and 1x0.5 cm.

o Skin involvement: Free grossly

- Distance from margins: 0.5 cm from least lateral margin.

- Frozen section:

-Lymph nodes : Free (0/6), six more nodes are left for paraffin examination.

- Suspicious medial and deep margins

- Free other margins.

Microscopic examination:

- First wired area:

- Histological type: Invasive duct carcinoma.

- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).

Verified by :

Dr.Heba Hasan

Patient Name : الأستاذة / ايمان محمد محمود سليم
Patient No. : 27605010101744
Path No. : 1246/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : اورام - دار خديعة - ميثاقه
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Tumor cellularity: 20%.
- Angiolysis: Not detected.
- Ductal carcinoma in situ: Not detected.
- Second wired area :
- Only ductal carcinoma component, intermediate grade, (5%).
- There is no evidence of invasive component
- Lymph node involvement: Positive (1/12), with some showed therapy effect.
- Frozen nodes (1/6).
- Paraffin nodes (0/6).
- Resection Margins: Free all resected margins and extra margins.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, wide local excision and axillary clearance specimens:

- Invasive duct carcinoma , grade II.
- Tumor cellularity: 20%.
- Free all resected margins and extra margins.
- Lymph nodes: Positive (1/12).
- Pathologic partial response.
- Stage : ypT1b , ypN1.
- RCB : 2.767 , RCB class II
- Comment: There is a disparity between frozen and paraffin results, however final diagnosis depends only on paraffin results.

Prof. Gamal Abdelatty
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 2 of 3

Printing By : marwa mohamed yoo



Patient Name	: الأستاذة / سمية بيومي عبد الله محمد	Request No.	: 13002998
Patient No.	: 27807092101901	Request Date	: 05/12/2024
Path No.	: 4780/24	Verification Date	: 15/12/2024
Sex	: Female	Financial Category	: اورام خارج السلام - مياتر
Age	: 46 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 5/12/2024.

Gross Examination:
- 4 grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Focal.
* Perineural invasion: No.
* Angiolymphatic invasion: No.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with focal DCIS.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR. Menna Nabil

Printing Date : 08/04/2025 12:57:08

Page 1 of 1

Printing By : thermal





Name: Mrs. Eman Mohamed Mahmoud Selim

Date: 9/11/2024

ID: 2224069566

Acc: 5047373

Referring Hospital: Azam Dar Al-Salam

Te-99m MDP whole body bone scan

Technique:

Whole body images were acquired 180 minutes after IV injection of 15 mCi of the tracer.

Findings:

Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.

Degenerative changes of the spine are seen.

Arthritic knee changes are noted.

Both kidneys are visualized.

Opinion:

No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandool, MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Model No. AM





Patient Name: Eman Mohamed Mahmoud Selim	Date : 25/11/2024
Exam : C.T NECK	Age : 48 Year(s)
Medical ID: 3087/2024	Pat. ID: 77240

NON CONTRAST MULTI-SLICE CT SCAN OF THE NECK: -

- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilaginous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal appearance of the neck muscles & vessels.
- Multiple small deep cervical lymph nodes noted, being subcentimetric in short axis.

Opinion:

- Multiple small deep cervical lymph nodes, likely inflammatory than neoplastic for clinical correlation & follow up.

Dr. Sherif Sameh, M.D.



Patient Name: Eman Mohamed Mahmoud Selim	Date : 24/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 3087/2024	Pat. ID: 77240

**COMBINED BILATERAL BREAST MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND OF BOTH BREASTS**

Indication:

Multifocal right breast pathologically proven malignant masses.

Received NACT

Report:

- Scattered fibro glandular density (ACR B)
- Stable right breast show UOQ multiple spiculated irregular mass lesion, of high density. The area involving the masses measures about 66x65x110mm. Two of which are marked by metallic clip.
- Complementary ultrasound revealed right breast multiple irregular spiculated hypoechoic mass lesions are seen from 10O'clock to 12o'clock, 2mm from skin and nearest one from nipple about 36mm, the largest one measuring about 15x10mm.
- No left suspicious micro-calcifications or spiculated masses.
- No left solid or cystic mass lesion.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Still seen one indeterminate right axillary lymph nodes are seen with diffuse cortical thickening about 3.3mm and preserved fatty hilum.
- Nonspecific left axillary LN.

Opinion:

- Right breast UOQ pathologically proven malignant masses with still noted ipsilateral indeterminate axillary lymph node showing overall stationary course (BIRADS 6).
- Normal study of the left breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Patient ID: 00559	Patient Name: Eman Mohamed Mahmoud Selim
Ref: 00559	09/04/2025 10:47:53 AM Report Date

Patient Name: Eman Mohamed Mahmoud Selim

Date: 09/04/2025

Stereotactic Guided Wire Localization:

Clinical summary: Status post NACT for pre-operative localization of inserted clips.

Technique:

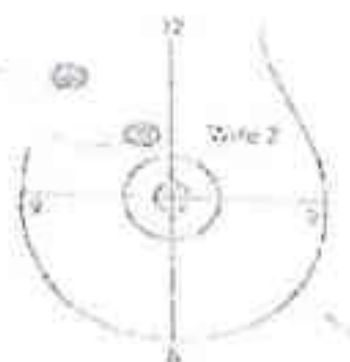
Under complete sterile conditions, 3ml of local anesthetic were administered at the site of concern in the RT breast. Two 21G wires were used to localize the inserted clips.

The wires are located at the right upper outer quadrant.

Wire 1: was inserted perpendicular to the peripherally located inserted clip.

Wire 2: was inserted perpendicular to the anteriorly located clip that was at 5.7 cm from the nipple with the hook seen anterior to the clip by 3.5mm.

Wire 1



Much Obligor
Dr. Sherihan Fakhry, MD.



الجمهورية العربية السورية
وزارة الداخلية

لبنان
محمد محمود سلوم
ابن شكرى سعد
القصور - القلوبية



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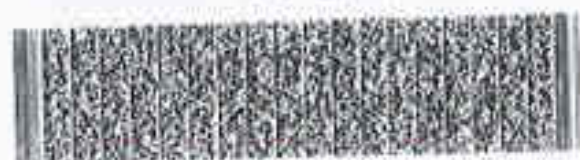
حاصله على ابلوم مسليح

ابن مسليح



البطاقة سارية حتى ٢٠٢٧/٠٩/٢٢

١١



Patient's Name:

Iman Mohammed Mahmoud

Date:

2026-06-03

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Status post right CBS & ALAND, followed by radio therapy, routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ, architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound** demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- axillary tail small circumscribed nodule with macrocalcification. No speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated axillary tail small benign looking nodule measuring 6 x 3 mm, no suspicious solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-Therapeutic sequelae.

Left breast (BIRADS 2): Axillary tail benign looking nodule.

Recommendation:-

For annual follow up.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Categories & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %
0 %
2 %
30 %

98-100%

MOHAMMED MOHAMMED MAHMOUD
02472 413026

Bilateral high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

References

Reluctant breast.

- ultrasound demonstrated UOQ posterior third area of parenchymal disruption underneath the postaxial scar with no solid, cystic lesions or fluid collections.
 - thickened skin reaching 5 mm and coarsen trabeculae.
 - normal nipple/areola complex.
 - clear right axilla. No pathological intra or supra-clavicular lymph nodes
- Dr. J. J. J. J. J.
- ultrasound demonstrated no solid or cystic lesions.
 - normal skin/nipple/areola complex.
 - no ipsilateral axillary LNs.

1000-0000/01/0000-0000\$10.00/0

Fig. 1: breast (BIRADS 2): Post-therapeutic sequelae, left breast (BIRADS 1): Normal study.

Recommendation:

Senior Cultural Fellow, U.S.

Best Records

© 1994, Westview Press, Inc.

20. All direct section of breast lumps, as well as the Axillary Tissue of Apollos, used for breast assessment combined with the Breast Health Care.

Exposure to a high concentration of

2.2.2.5. *Access to information study*

Sustained and Canceled Trials

100% of a normal study, routine screening	0.5%
100% of a benign findings, routine screening	0.1%
100% of a probably benign, for close follow up	2.5%
100% of a probably malignant for biopsy	50.5%

Scientific Information,

5676

the following conditions:

to blood sampling.

CONTACT: 516-471-2200, for info@nyu.edu

010423-3 Pathology governs maturation

3B-1007E



الجمهورية الجزائرية الديمقراطية الشعبية
بموجب المرسوم رقم 17/01 المؤرخ 17/01/2017

الاسم: محمد بن عبد الله
اللقب: بن عبد الله
الكنية: -



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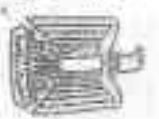
100221630



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الاسم: محمد بن عبد الله

اللقب: بن عبد الله

الكنية: -



NAME EMAN MOHAMED MAHMUD
DATE 4-1-2026

Bilateral high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

Right breast:

- Breastound demonstrated UOD posterior third are of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla, No pathological infra or supra-clavicular lymph nodes

Left breast:

- Breastound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- No suspicious axillary LNs.

Conclusion:

Right breast (SIRADS 2): Post-therapeutic sequelae.

Left breast (SIRADS 1): Normal study.

Recommendation:

3-6 Annual follow up.

Best Regards,
Dr. Eman Mahmoud, MD

Medical classification of breast lesions according to the American College of Radiology used for breast assessment combined with

<u>Selected Recommendations</u>	<u>Selected Cancer Risk</u>
<u>Category A: Recommendations</u>	
1. USG or Mammography study.	
2. USG or Mammography study, routine screening	0%
3. USG or Mammography study, routine screening	0%
4. USG or Mammography study, routine screening	2%
5. USG or Mammography study, routine screening	50%
6. Mild suspicion.	
7. Moderate suspicion.	
8. High suspicion.	
9. USG or Mammography study, routine screening	98-100%
10. USG or Mammography study, routine screening	

Patient Name	: Mr. James Earl Smith
Patient No.	: 27605D10101744
Path No.	: 1246725
Sex	: Male
Age	: 48 Y
Referring Doc :	

Request No. : 13005623
Request Date : 08/04/2025
Verification Date : 29/04/2025
Financial Category : *مريض - عائله*
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Flight breast cancer post NACT.

Nature of the specimen:

Region wide local excision and auxiliary clearance specimens, collected at 9/4/2025.

Gross Examination

Five specimens were recaptured.

*Auxiliary clearance specimen : Two Fibrofatly tissue patches measured 5x3.5 cm and 4x3 cm, enclosing 12 needles up to 2.5x2 cm.

Extra medial margin: Fibrofatty tissue piece measured 3x2.5 cm

Extra donor margin: Fibroblast tissue piece measured 2.5x2 cm

Figure 1. Model of a 10 × 10 cm speedometer dialscale. 100 mm cm with covering and ellipse translated 90.5 cm.

- Tumor extent

c Number: Two (wired) in defined rubbery greyish white areas.

Size: 1.5x1cm and 1x0.5 cm.

D Skin involvement: Free grossly

Distance from margins: 0.5 cm from least lateral margin

Frozen section:

Lymph nodes: Free (0/6), six more nodes are left for paraffin examination.

- Deep margins
- Deep margins
- Deep margins

Microscopic examination:

- First wired area:

Histological type: Invasive duct carcinoma

* Grade (differentiation + nuclear pleomorphism + mitosis): G II(3+2+1).

Verified by:

Dr. Heba Hassan

Printed Date: 30-01-2015 11:25:19

Page 1 of 3

Painting By: mervat mohamed youssef

ECHO REPORT

NAME: محمد بن علي DATE: 29/3/2026
ID NUMBER: / SEX: ♀ AGE:

EF	<u>66%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.5</u>	3.5-5.8cm	AO.R	<u>2.3</u>	3-3.7cm
LVEDSD	<u>2.3</u>	2.4cm	LA	<u>3.6</u>	2-4cm
FS	<u>30%</u>	25-45%	RV		0.7-2.3cm(supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RumA
LA and Ao Root Dimensions Normal
Mitral Valve Normal
Aortic Valve Normal
Tricuspid Valve Normal
Pulmonary Valve Normal
Right Side and Systolic Function Normal
Intracardiac Masses and Effusion None

DR.

[Signature]

MUHAMMAD EMMAN MOHAMMED MAHMOUD
DA 73 4-1-2026

Bilateral high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study

Findings:

Right breast:

- Ultrasound demonstrated UOQ posterior third area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
 - Thickened skin reaching 5 mm and coarsen trabeculae.
 - Normal nipple/areola complex.
 - Clear right axilla. No pathological infra or supra-clavicular lymph nodes.
- Left breast:
- Ultrasound demonstrated no solid or cystic lesions.
 - Normal skin/nipple/areola complex.
 - No-specific axillary LNs.

Conclusion:

Right breast (BIRADS 2): Post-therapeutic sequelae.
Left breast (BIRADS 1): Normal study.

Recommendation:

Surveillance follow up.

Best Regards,
DR. EMMAN JEMMED, M.D

Bibliography: Identification of breast lesions sensitive due to the American College of Radiology used for breast assessment combined with ultrasonography.

Cancer Recommendations:

01-03-2018 Informative study.

Suggested Cancer Risk

BIRADS 1: Normal study, resulting screening
survival to benign findings, routine screening
survival to probably benign, for close follow up.
BIRADS 2: Probably malignant, for biopsy.

0 %
0 %
2 %
50 %

As a rule, suspicion,
a) moderate suspicion,
b) high suspicion.

BIRADS 3: Malignant, for biopsy.
BIRADS 4: Pathologically proven malignant.

99-100%

Referring Doc:	Ref: jms 200 1011744
Age	: 48 y
Sex	: Female
Path No.	: 1246/25
Patient Name	: 27605010101744

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : **مستشفى - مستشفى**
Department : Out Patient
Registered by : Mrs Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Eight wide local excision and axillary clearance specimens, collected at 9/4/2025

Gross Examination

Five specimens were recaptured.

•Auxiliary clearance specimen : Two Fibrofatly tissue pelces measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm .

Extra-medial margin: Fibrofatty tissue piece measured 3x2.5 cm

Extra deep margin: fibrotic tissue piece measured 2.5x2 cm

1000 V and 100 nA. The positive electrode (cathode) was covered with an ellipse measured 0x3.6 cm.

Turner, J. 1990.

Number Two (wired) in defined rubbery greyish white areas.

Size: 1.5X1X1cm and 1X0.5 cm.

© Seán involvements: Free grossly

Distance from margins: 0.5 cm from least lateral margin.

Frozen section:

Lyophil nodes : Free (0/6), six more nodes are left for paraffin examination.

Free other margins.

Microscopic examination:

First wired area:

Histological type: Invasive duct carcinoma.

- Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+1)

Verified by:

Dr Heba Hussen

Printing Date: 30/04/2015 11:25:19

Page 1 of 3

Printed by: ameer muhammad yous

Patient Name : **أ/م/ب/ج/د/هـ/و/ز/ح/ط/ي/ك/ل/م/ن/س/ع/ف/ق/ص/غ/ظ/ف/ي/ك/ل/م/ن/س/ع/ف/ق/ص/غ/ظ**
Patient No. : 27605010101744
Fath No. : 1246/25
Sex : Female
Age : 48 Y
Refereing Doc :

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : **مستشفى - مريض**
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data:
Right breast cancer post NACT.

Nature of the specimen:
Right wide local excision and axillary clearance specimens , collected at 9/4/2025.

Gross Examination:

Five specimens were received:

- Axillary clearance specimen : Two Fibrofatty tissue peices measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm .
- Extra medial margin:Fibrofatty tissue piece measured 3x2.5 cm
- Extra deep margin: Fibrofatty tissue piece measured 2.5x2 cm
- Right wide local excision specimen measured 10.8x4 cm with covering skin ellipse measured 9x3.5 cm.
- Tumor extent
- o Number: Two (wired) ill defined rubbery greyish white areas .
- o Size: 1.5x1x1cm and 1x0.5 cm.
- o Skin involvement: Free grossly
- Distance from margins: 0.5 cm from least lateral margin.
- Frozen section:
- Lymph nodes : Free (0/6), six more nodes are left for paraffin examination.
- Suspicious medial and deep margins
- Free other margins.

Microscopic examination:

- First wired area:
- Histological type:Invasive duct carcinoma,
- Grade (differentiation+ nuclear pleomorphism + mitosis) : G II(3+2+1).

Verified by :
Dr.Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 1 of 3

Printing By : marwa mohamed youss

CAP
accredited

مركز - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Almer El Qadema - Cairo, Egypt

ECHO REPORTNAME: أحمد بن عليDATE: 29 / 3 / 2026ID NUMBER: /SEX: ♀ AGE:

EF	<u>66%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.5</u>	3.5-5.8cm	AO.R	<u>2.3</u>	3-3.7cm
LVEDS	<u>2.3</u>	2-4cm	LA	<u>3.6</u>	2-4cm
FS	<u>30%</u>	25-45%	RV		0.7-2.3cm(supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions NormalLV Wall Thickness NormalLV Diastolic Function NormalSWMA No RumALA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function TAPE: 2.3

Intracardiac Masses and Effusion

DR.





ERM03672BS45



100221650



Ministry of Health and Wellness
Kingdom of Saudi Arabia

وزارة الصحة
المملكة العربية السعودية



MDT form

1- Patient ID
1- Name : Abdulaziz bin Abdulaziz

Sex :

2- Medical number:
Patient ID :

03231138
Old MED :

03/04/2024
2023-03-15

3- ID number : 27603010101744

Patient Age : 45 Y



Family history	Free history	Phone	Age	BMI	Sex	REASON OF MDT
Medical status	Free history	MRSA status	MRSA status	GROUP	GROUP	Date of MDT
CR medications	CR medications	MRSA status	MRSA status	MRSA status	MRSA status	2023-03-15

Clinical data

Breast Imaging :

Mets Workshop :

1- Breast UOQ multiple well-circumscribed irregular mass lesion from 1.5 cm to 1.2 cm, from 1.5 cm, nearest size from nipple 2.0 mm target
date 24/12/2023
date 17/12/2023
date 17/12/2023

ER	PR	HER 2	SI-SH	KIR
0/0	5/5	3	3	4
0	0	0	0	0

Sonography
CESM: UOQ INDEX LESION 24MM, 2 OTHER LESIONS, IPSILATERAL PATH AXILLA

PATIENT HISTORY
received NACT TCHP x 6 cycles
RT BCS+AD: free margin
VPT 1m largest diameter (1.5CM) LN 1/12
partial pathological response

MDT OUTCOME & PLAN

adjuvant Kadcyla x12
port
Adjuvant AI

SIGNATURE

MEDICAL ONCOLOGY	RADIOLOGY	PATHOLOGY	PSYCHIATRY
<u>Dr. Ali</u>	<u>Dr. Ali</u>	<u>Dr. Ali</u>	<u>Dr. Ali</u>
<u>Dr. Ali</u>	<u>Dr. Ali</u>	<u>Dr. Ali</u>	<u>Dr. Ali</u>



Ministry of Health and Family Welfare
Government of Sindh



MDT form

1. Patient ID: 10000000000000000000

Sex:

2. Medical number:

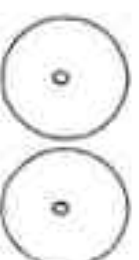
00000000000000000000

Date Recd:

3. ID number:

Patient Age: 40 Y

E M D T



Family history: No history of breast cancer in family

Menstrual status: Menstruating regularly

Co-morbidity: No co-morbidity

Reason for MDT: Breast lump

Date of MDT: 05/10/2024

Clinical data

Meta Workshop:

Free

Breast Imaging: Ultrasound shows a 2x2 cm irregular mass lesion from 10 o'clock to 2 o'clock from skin, nearest one from nipple 26 mm largest one 24 x 17 mm. BI-RADS 5.

None of the following axillary LN

EC	ER	PR	HER 2 Neu	SLISH	HER 2
100	100	100	100	100	100
2	0	0	0	0	0

Sonultramamography

PATIENT HISTORY

FOR CSM THEN REMDT
AND RT AXILARY BIOPSY

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	RADIOLOGY	PATHOLOGY	NUTRITION
Signature	Signature	Signature	Signature
Signature	Signature	Signature	Signature

Patient Name : السيدة / أمينة محمد حسن
 Patient No. : 27605010101744
 Path No. : 1246125
 Sex : Female
 Age : 48 Y

Request No. : 13005623
 Request Date : 09/04/2025
 Verification Date : 29/04/2025
 Financial Category : السيدات
 Department : Out Patient
 Registered by : Mrs. Mayada

Referring Doc :

Pathology Report

Clinical Data:
 Right breast cancer post NACT, -----

Nature of the specimen:
 Right wide local excision and axillary clearance specimens, collected at 9/4/2025.

Gross Examination:
 Five specimens were received:

- Axillary clearance specimen : Two Fibrofatty tissue pieces measured 5x2.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.
- Extra medial margin: Fibrofatty tissue piece measured 2.5x2 cm
- Extra deep margin: Fibrofatty tissue piece measured 10.5x7.4 cm with covering skin ellipse
- Right wide local excision specimen measured 9x3.5 cm.
- Tumor extent:
- o Number: Two (wired) ill defined rubbery greyish white areas.
- o Size: 1.5x1x1cm and 1x0.5 cm.
- o Skin involvement: Free grossly
- o Distance from margins: 0.5 cm from least lateral margin.
- Frozen section:
- o Lymph nodes : Free (0/6), six more nodes are left for paraffin examination.
- Suspicious medial and deep margins.
- Free other margins.

Microscopic examination:

- First wired area:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+1)

Verified by :
 Dr. Heba Hasan

Printing By : marwa mohar

Printing Date : 30/04/2025 11:25:19

Page 1 of 3

طريق رقم (4) - طبر - الطب - مستشفى رقم (987) - طبر - مصر الجديدة - القاهرة - مصر
 Station No. (4) - First Floor - Building No. 987 - Nak Corniche - Mistr El Qudous - Cairo, Egypt

CAP

Patient Name	: John James Jones / 123456
Patient No.	: 27805010101744
Path No.	: 383524
Sex	: Female
Age	: 48 Y
Referring Doc :	

Request No.	: 13007115
Request Date	: 08/10/2024
Verification Date	: 24/10/2024
Financial Category	: لزم - ملزم
Department	: Out Patient
Registered by:	: Marwa mohamed wassan

Pathology Report

Clinical Data: Known case of right breast cancer, enlarged right axillary lymph nodes.
Nature of Specimen: Tru-cut biopsy, collected at 08/10/2024.

Gross Examination:

- Multiple gray/ish white tissue cores up to 0.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for GATA3, ER, PR, Her2, GATA3 clone (L50-823), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Microscopic Examination:

Histopathological examination of sections prepared from the lymph node biopsy received reveals complete effacement of the normal nodal architecture. There is replacement by malignant epithelial neoplasm formed of sheets and irregular tubules of malignant epithelial cells. The malignant cells exhibit moderate degree of anaplasia.

• IPT (Positive control showed appropriate reactivity):

GATA3 (Stained control showed appropriate reactivity):

- GATA3: Strong diffuse nuclear reaction of tumor cells.
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
- Her2: Strong complete membrane reaction in >10 % of tumor cells (score 3+).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Known case of right breast cancer with enlarged right axillary lymph nodes, Tru-cut biopsy: Metastatic adenocarcinoma of breast origin.

PT:

- ER: Positive (score 8/8)
- PR: Positive (score 5/8)

Verified by:

DR. NERMEEN MOHAMMAD

Printing Date : 24/10/2024 11:21:54

Page 1 of 2

Printing By : mawa mohammed yousse

Dr. CAP
KORHONEN

عبد الله بن عبد الله - Egypt - 307 - 308 - 309 - 310 - 311 - 312 - 313 - 314 - 315 - 316 - 317 - 318 - 319 - 320 - 321 - 322 - 323 - 324 - 325 - 326 - 327 - 328 - 329 - 330 - 331 - 332 - 333 - 334 - 335 - 336 - 337 - 338 - 339 - 340 - 341 - 342 - 343 - 344 - 345 - 346 - 347 - 348 - 349 - 350 - 351 - 352 - 353 - 354 - 355 - 356 - 357 - 358 - 359 - 360 - 361 - 362 - 363 - 364 - 365 - 366 - 367 - 368 - 369 - 370 - 371 - 372 - 373 - 374 - 375 - 376 - 377 - 378 - 379 - 380 - 381 - 382 - 383 - 384 - 385 - 386 - 387 - 388 - 389 - 390 - 391 - 392 - 393 - 394 - 395 - 396 - 397 - 398 - 399 - 400 - 401 - 402 - 403 - 404 - 405 - 406 - 407 - 408 - 409 - 410 - 411 - 412 - 413 - 414 - 415 - 416 - 417 - 418 - 419 - 420 - 421 - 422 - 423 - 424 - 425 - 426 - 427 - 428 - 429 - 430 - 431 - 432 - 433 - 434 - 435 - 436 - 437 - 438 - 439 - 440 - 441 - 442 - 443 - 444 - 445 - 446 - 447 - 448 - 449 - 450 - 451 - 452 - 453 - 454 - 455 - 456 - 457 - 458 - 459 - 460 - 461 - 462 - 463 - 464 - 465 - 466 - 467 - 468 - 469 - 470 - 471 - 472 - 473 - 474 - 475 - 476 - 477 - 478 - 479 - 480 - 481 - 482 - 483 - 484 - 485 - 486 - 487 - 488 - 489 - 490 - 491 - 492 - 493 - 494 - 495 - 496 - 497 - 498 - 499 - 500 - 501 - 502 - 503 - 504 - 505 - 506 - 507 - 508 - 509 - 510 - 511 - 512 - 513 - 514 - 515 - 516 - 517 - 518 - 519 - 520 - 521 - 522 - 523 - 524 - 525 - 526 - 527 - 528 - 529 - 530 - 531 - 532 - 533 - 534 - 535 - 536 - 537 - 538 - 539 - 540 - 541 - 542 - 543 - 544 - 545 - 546 - 547 - 548 - 549 - 550 - 551 - 552 - 553 - 554 - 555 - 556 - 557 - 558 - 559 - 560 - 561 - 562 - 563 - 564 - 565 - 566 - 567 - 568 - 569 - 570 - 571 - 572 - 573 - 574 - 575 - 576 - 577 - 578 - 579 - 580 - 581 - 582 - 583 - 584 - 585 - 586 - 587 - 588 - 589 - 590 - 591 - 592 - 593 - 594 - 595 - 596 - 597 - 598 - 599 - 600 - 601 - 602 - 603 - 604 - 605 - 606 - 607 - 608 - 609 - 610 - 611 - 612 - 613 - 614 - 615 - 616 - 617 - 618 - 619 - 620 - 621 - 622 - 623 - 624 - 625 - 626 - 627 - 628 - 629 - 630 - 631 - 632 - 633 - 634 - 635 - 636 - 637 - 638 - 639 - 640 - 641 - 642 - 643 - 644 - 645 - 646 - 647 - 648 - 649 - 650 - 651 - 652 - 653 - 654 - 655 - 656 - 657 - 658 - 659 - 660 - 661 - 662 - 663 - 664 - 665 - 666 - 667 - 668 - 669 - 670 - 671 - 672 - 673 - 674 - 675 - 676 - 677 - 678 - 679 - 680 - 681 - 682 - 683 - 684 - 685 - 686 - 687 - 688 - 689 - 690 - 691 - 692 - 693 - 694 - 695 - 696 - 697 - 698 - 699 - 700 - 701 - 702 - 703 - 704 - 705 - 706 - 707 - 708 - 709 - 710 - 711 - 712 - 713 - 714 - 715 - 716 - 717 - 718 - 719 - 720 - 721 - 722 - 723 - 724 - 725 - 726 - 727 - 728 - 729 - 730 - 731 - 732 - 733 - 734 - 735 - 736 - 737 - 738 - 739 - 740 - 741 - 742 - 743 - 744 - 745 - 746 - 747 - 748 - 749 - 750 - 751 - 752 - 753 - 754 - 755 - 756 - 757 - 758 - 759 - 760 - 761 - 762 - 763 - 764 - 765 - 766 - 767 - 768 - 769 - 770 - 771 - 772 - 773 - 774 - 775 - 776 - 777 - 778 - 779 - 780 - 781 - 782 - 783 - 784 - 785 - 786 - 787 - 788 - 789 - 790 - 791 - 792 - 793 - 794 - 795 - 796 - 797 - 798 - 799 - 800 - 801 - 802 - 803 - 804 - 805 - 806 - 807 - 808 - 809 - 810 - 811 - 812 - 813 - 814 - 815 - 816 - 817 - 818 - 819 - 820 - 821 - 822 - 823 - 824 - 825 - 826 - 827 - 828 - 829 - 830 - 831 - 832 - 833 - 834 - 835 - 836 - 837 - 838 - 839 - 840 - 841 - 842 - 843 - 844 - 845 - 846 - 847 - 848 - 849 - 850 - 851 - 852 - 853 - 854 - 855 - 856 - 857 - 858 - 859 - 860 - 861 - 862 - 863 - 864 - 865 - 866 - 867 - 868 - 869 - 870 - 871 - 872 - 873 - 874 - 875 - 876 - 877 - 878 - 879 - 880 - 881 - 882 - 883 - 884 - 885 - 886 - 887 - 888 - 889 - 890 - 891 - 892 - 893 - 894 - 895 - 896 - 897 - 898 - 899 - 900 - 901 - 902 - 903 - 904 - 905 - 906 - 907 - 908 - 909 - 910 - 911 - 912 - 913 - 914 - 915 - 916 - 917 - 918 - 919 - 920 - 921 - 922 - 923 - 924 - 925 - 926 - 927 - 928 - 929 - 930 - 931 - 932 - 933 - 934 - 935 - 936 - 937 - 938 - 939 - 940 - 941 - 942 - 943 - 944 - 945 - 946 - 947 - 948 - 949 - 950 - 951 - 952 - 953 - 954 - 955 - 956 - 957 - 958 - 959 - 960 - 961 - 962 - 963 - 964 - 965 - 966 - 967 - 968 - 969 - 970 - 971 - 972 - 973 - 974 - 975 - 976 - 977 - 978 - 979 - 980 - 981 - 982 - 983 - 984 - 985 - 986 - 987 - 988 - 989 - 990 - 991 - 992 - 993 - 994 - 995 - 996 - 997 - 998 - 999 - 1000 - 1001 - 1002 - 1003 - 1004 - 1005 - 1006 - 1007 - 1008 - 1009 - 1010 - 1011 - 1012 - 1013 - 1014 - 1015 - 1016 - 1017 - 1018 - 1019 - 1020 - 1021 - 1022 - 1023 - 1024 - 1025 - 1026 - 1027 - 1028 - 1029 - 1030 - 1031 - 1032 - 1033 - 1034 - 1035 - 1036 - 1037 - 1038 - 1039 - 1040 - 1041 - 1042 - 1043 - 1044 - 1045 - 1046 - 1047 - 1048 - 1049 - 1050 - 1051 - 1052 - 1053 - 1054 - 1055 - 1056 - 1057 - 1058 - 1059 - 1060 - 1061 - 1062 - 1063 - 1064 - 1065 - 1066 - 1067 - 1068 - 1069 - 1070 - 1071 - 1072 - 1073 - 1074 - 1075 - 1076 - 1077 - 1078 - 1079 - 1080 - 1081 - 1082 - 1083 - 1084 - 1085 - 1086 - 1087 - 1088 - 1089 - 1090 - 1091 - 1092 - 1093 - 1094 - 1095 - 1096 - 1097 - 1098 - 1099 - 1100 - 1101 - 1102 - 1

د. مهنى احمد سامى
مركز البحوث والتكنولوجيا
المركز الطبي المتكامل



د. مهنى علي الشبلي
مركز البحوث والتكنولوجيا
المركز الطبي المتكامل

Name: Mrs. Eman Mohamed Mahmoud Selim
Date: 9/11/2024
Referring Hospital Awram Dar Al-Salaam.

ID:2224069566

Acc.:5047373

Tc-99m MDP whole body bone scan

Technique:

- Whole-body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandeel, MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by AR



Patient Name	: اسم المريض / اسم المريض	Request No.	: 13005623
Patient No.	: 27605010101744	Request Date	: 09/04/2025
Path No.	: 1246/25	Verification Date	: 29/04/2025
Sex	: Female	Financial Category	: ١٠٠٠ - ١٠٠٠
Age	: 48 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs. Mayada Hegazy

Pathology Report

- Tumor cellularity: 20%.
- Angioinvasion: Not detected
- Ductal carcinoma in situ: Not detected
- Second wired area :
- Only ductal carcinoma component, intermediate grade, (5%).
- There is no evidence of invasive component
- Lymph node involvement Positive (1/12), with some showed therapy effect.
- Frozen nodes (1/6).
- Paraffin nodes (0/6).
- Resection Margins: Free all resected margins and extra margins.
- Histologic response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, wide local excision and axillary clearance specimens;

-Invasive duct carcinoma , grade II.

-Tumor cellularity: 20%.

-Free all resected margins and extra margins.

-Lymph nodes: Positive (1/12)

-Pathologic partial response.

-Stage : ypT1b , ypN1

-RCB : 2.767 , RCB class II

Comment: There is a disparity between frozen and paraffin results, however final diagnosis depends only on paraffin results.

Prof. Gamal Abdelatty

Dr. Menna Nabil

Verified by :

Dr. Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 2 of 3

Printing By : marwa mohamed youss





Patient Name : **المنيرة محمد**
Patient No. : 27605010101744
Path No. : 3835/24
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13007115
Request Date : 09/10/2024
Verification Date : 24/10/2024
Financial Category : **مريض** - Out Patient
Department :
Registered by : marwa mohamed youss

Pathology Report

- Her2: Positive (score 3+).
- Ki67: Positive in 10% of tumor cells.

• Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none, 1, weak, 2, intermediate, and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013, IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faintly/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faintly/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells

Signature
Dr. Wajaa Ghonaim,
Dr.Nermiin Mohamed

Verified by :
DR.Nermoen Mohammed

Printing Date : 24/10/2024 11:31:54

Page 1 of 2

Printing By : marwa mohamed youss





Patient Name: Erian Mohamed Mahmoud		Date : 07/08/2024	
Sex: M		Age : 48 Year(s)	
Exam : MAMMOGRAPHY BOTH		Pat. ID: 77240	
Medical ID: 27605010101744			

Combined bilateral breast mammography and complementary ultrasound

Indication:

right breast palpable lump.

Positive family history (her mum).

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ multiple spiculated irregular mass lesion, of high density.
- Complementary ultrasound revealed right breast multiple irregular spiculated hypoechoic mass lesions are seen from 10o'clock to 12o'clock, with internal vascularity detected within on color Doppler, seen 2mm from skin and nearest one from nipple about 36mm, the largest one measuring about 24x17mm, consistent to the palpable lump.
- No left solid or cystic mass lesion.
- Normal skin thickness, and nipple-areola complex of the left breast.
- One indeterminate right axillary lymph nodes are seen with diffuse cortical thickening about 3.3mm and preserved fatty hilum.
- Nonspecific left axillary LN.

Opinion:

- Right breast UOQ multiple malignant looking mass lesions (BIRADS 5) for histopathological correlation.
- One right indeterminate axillary LNs.
- Benign findings left breast (BIRADS 2)

Signature Dr. Nehal Hamdy

07/08/2024 1:41 PM

- No speculated masses, asymmetry, distortion or suspicious calcifications.
- Recombined (post contrast) images show: no pathological uptake.
- Ultrasound demonstrated UOQ 3.00 oval isoechoic lesion measures 7x3mm.
- No cystic masses.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast : 10-12.00 multifocal malignant process with finding as described (BIRADS6) .

Left breast : UOQ probably benign non enhancing mass lesion (BIRADS3) for 6 months follow up.

DR. Sharmistha Sen, MBBS, FRCS



دكتور Eman Kassem
أخصائية أمراض القلب والكلى
قسم أمراض القلب والكلى
1441 هـ

1000

Name: Doaa Fathy

Echo report

Date: 11/5/2025

age: 23Y

EF	71%	54-74%	PWd	0.8	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO R	2.5	2-3.7cm
LVEDS	2.9	2-4cm	LA	2.4	2-4cm
FS	41	25-45%	RV		0.7-2.3cm(supine)
IVSd	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

Normal LV internal dimensions and systolic function (EF=71%) by M-mode.

Reversed mitral inflow pattern.

No resting SWMA could be detected.

Normal LA size.

Mitral Valve: Normal valve with trivial MR.

Aortic valve: Normal valve and flow.

Tricuspid valve: Normal valve with trivial TR.

Right side and systolic function: Normal by TDI.

No intracardiac masses or thrombi.

Normal pericardium with no pericardial effusion.

Normal IVC diameter and collapsibility.

Conclusion:

- Normal LV internal dimensions and systolic function.
- Left ventricular diastolic dysfunction grade I.

Dr Eman Kassem, MD
Cardiology

Signature

Presidential Initiative for Women's Health - Pathology Lab
 الطباعة الإلكترونية لخدمة المرأة - معمل الفحوصات النسائية

Signature

Patient Name	: اسم المريض / Patient Name	Request No.	: 13004542
Patient No.	: 27707060103141	Request Date	: 13/05/2025
Path No.	: 1733/25	Verification Date	: 29/05/2025
Sex	: Female	Financial Category	: مريض - Patient
Age	: 47 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youssef

Pathology Report

Clinical Data: Left breast cancer post NACT.

Nature of the specimen: Left breast & Left axilla (MRM)(Post NACT)

Gross Examination:

- Procedure: Left MRM measured 17x13x3 cm with covering skin ellipse measured 17x10cm.
- Localization: LOQ
- Tumor extent: A greyish white tumour bed site
- Size: 1.8 x1.5 x1.5 cm.
- Skin involvement: Nipple & areola were unremarkable grossly.
- Distance from margins: 2 cm away from deep margin
- Axilla: Fibrofatty tissue piece measured 5x5x2 cm entangling 13 nodules up to 1.5x1 cm
- Microscopic examination:
- Histological type: Residual invasive lobular carcinoma. Cellularity 30%
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Lobular carcinoma in situ: Focal
- Lymphovascular emboli: Positive
- Skin involvement: Free nipple and areola
- Lymph node involvement: Positive L.Ns (5/13)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic partial response

Diagnosis:

- Left MRM (post NACT):
- Residual invasive lobular carcinoma grade II with focal LCIS
- Free deep margin
- Free nipple and areola
- Positive L.Ns (5/13)

Verified by:

Dr.Heba Hasan

Printing Date : 05/06/2025 12:17:59

Page 1 of 1

Printing By : marwa mohamed youssef



Imaging report



Name : Mrs. Doaa Farhy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

F-18 FLUORODEOXYGLUCOSE (FDG) PET/CT OF THE WHOLE BODY:

◆ Clinical History:

- Metastatic workup of pathological proven left breast cancer.

◆ Procedure:

- Dose/Radiopharmaceutical: 8 mCi F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Blood glucose level: mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- A low-dose non-contrast CT images for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
- A diagnostic post-contrast CT examination of the same regions was performed after IV administration of non-ionic contrast.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.

◆ Findings

Head and Neck:-

- The brain appears normal on CT, with no abnormal focal metabolic activity detected.
- No pathologically enlarged or metabolically active cervical lymphadenopathy.
- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx, thyroid and salivary glands.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.

Imaging report



كايرو سكان
CAIRO SCAN
Scanning & USS

Name : Mrs. Doaa Farhy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

- No metabolically active or pathologically enlarged axillary, supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- Normal tracer biodistribution is noted at the myocardium and great vessels.

Abdomen and Pelvis:

- Liver is of average size, showing homogenous texture and smooth outline.
- No hepatic focal lesions or areas of abnormal enhancement or metabolic activity.
- No biliary radicles dilatations could be noted.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta and IVC as well as pelvic organs
- Normal FDG activity is seen at the gastrointestinal and genitourinary systems, with no striking focal hypermetabolic lesions detected.
- No pathologically enlarged or metabolically abdominal or pelvic lymph nodes.
- No ascites.

Musculo-skeletal:

- Solitary hypodense bony lesion is seen occupying D5 vertebral body with SUV max 4.



- Multiple tiny dense sclerotic foci are seen at the left iliac bone, few lumbo-sacral vertebrae mainly at D12 vertebral body.



Patient Name: Doaa Far'hy Hussein Ibrahim	Date : 22/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 3055/2024	Pat. ID: 78865

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows LOQ spiculated irregular mass lesion with overlying microcalcifications
- Complementary ultrasound revealed :
 - **Left breast:**
 - Left breast LOQ at about 4 o'clock irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, 1.5cm from the nipple, 0.3cm from the overlying skin, measuring 2.4x1.2 cm, consistent to the palpable lump.
 - Another two satellite lesions seen at 5 o'clock measured 12x9mm and 5-6 o'clock measured 7x4mm
 - Single pathological left axillary lymph node showing muffled hila, measuring 1.1x1.6cm, with cortical thickness reaching 5mm.
 - Free left supra and infraclavicular regions
 - **Right breast:**
 - No solid or cystic mass lesions
 - Nonspecific right axillary LN.

Opinion:

- Left breast LOQ multifocal malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Basma Mohamed
22/03/2025 2:39 PM

Patient Name: الأستاذة / د. هبة مصطفى
Patient No.: 27707060103141
Path No.: 2964/24
Sex: Female
Age: 47 Y
Referring Doc.

Request No.: 13002328
Request Date: 17/08/2024
Verification Date: 08/09/2024
Financial Category: لورم دى فلام - مريض
Department: Out Patient

Registered by: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 56% and 5, between 37%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Marwa Mahmoud,
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

Printing Date : 08/09/2024 10:25:21

Page 2 of 2

Printed By : marwa mohamed yousse





Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - مختبر الأمراض



Patient Name
Patient No. 27707060103141
Path No. 2964/24
Sex Female
Age 47 Y
Referring Doc.

Request No. 13002328
Request Date 17/08/2024
Verification Date 08/09/2024
Financial Category
Department Out Patient

Registered by marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Marwa Mahmoud.

Dr. Sagda Elkady.

Verified by :

Dr. Sagda El-Kady

Printing Date : 08/09/2024 16:25:21

Page 2 of 2

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Kousa No. (4) - First Floor-Building No. 307- Nile Corridor - Miter El Qadima - Cairo, Egypt

BQHEYA

(Baqheya Department)

بقرية

قسم الأشعة

PATIENT NAME: DOAA FATHY HUSEN IBRAHIM

GENDER: Female

DATE : 19-Sep-2024 11:28 AM

PATIENT ID: 10124249274

AGE: 47 y

ORDER: 3041678,3041679

**POST CONTRAST MULT-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE
LIVER**

TECHNIQUE:

- Multi-slice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 80 ml.

FINDINGS:

1-CHEST

- Left LOQ and retroareolar asymmetrical and enhancing parenchymal densities.
- Multiple pathological left level I, II and III axillary lymph nodes, the largest measures about 26x10mm.
- Few prominent left supraclavicular lymph nodes, the largest measures about 7x4mm.
- Few right middle and lower lung lobe calcified nodules likely post-granulomatous sequel.
- Left lower lung lobe thin atelectatic band.
- No sizable other pulmonary nodules, pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Right thyroid lobe hypodense nodule for US correlation.

2- ABDOMEN AND PELVIS

- The liver is enlarged with homogenous CT texture showing bi-lobar few

PATIENT: DOAA FATHY HUSEN IBRAHIM / 10124249274 / Female / 47 y / 19-Sep-2024 11:28 AM / 3041678,3041679 / 10124249274 / 11/09/2024 11:28 AM / 3041678,3041679 / 10124249274



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16602

قسم الأشعة - باقرية

الطبيب: د. أحمد محمد عبد الله

11/09/2024

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة
مركز التخصص في أمراض السرطان
مستشفى دار السلام
إدارة الأشعة

Patient Name: Doaa Farhy Hussein Ibrahim	Date : 08/09/2024
Exam : C.T ABDOMEN & PELVIS	Age : 47 Year(s)
Medical ID: 3055/2024	Pat. ID: 78865

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:

- Pathologically enlarged left axillary lymph nodes level I, II and III as well as left subpectoral, The largest at level I measuring 11 x 23 mm.
- Multiple discretely prominent right axillary lymph nodes are also noted.
- Middle lobe tiny calcific nodule measuring 3 mm. Otherwise, clear both lung fields. No sizable pulmonary nodules and no masses, consolidation or cavitory lesions.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Mildly enlarged liver with homogenous CT texture. No definite focal lesions on non contrast basis or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Imaging report

Name : Mrs. Doaa Far'hy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

F-18 FLUORODEOXYGLUCOSE (FDG) PET/CT OF THE WHOLE BODY:

◆ Clinical History:

- Metastatic workup of pathological proven left breast cancer.

◆ Procedure:

- Dose/Radiopharmaceutical: 8 mCi F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Blood glucose level: mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- A low-dose non-contrast CT images for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
- A diagnostic post-contrast CT examination of the same regions was performed after IV administration of non-ionic contrast.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed

◆ Findings

Head and Neck:-

- The brain appears normal on CT, with no abnormal focal metabolic activity detected.
- No pathologically enlarged or metabolically active cervical lymphadenopathy.
- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx, thyroid and salivary glands.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.

Imaging report



كايرو سكان
CAIRO SCAN
Imaging & Lab

Name : Mrs. Doaa Fathy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

- No metabolically active or pathologically enlarged axillary, supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- Normal tracer biodistribution is noted at the myocardium and great vessels.

Abdomen and Pelvis:

- Liver is of average size, showing homogenous texture and smooth outline.
- No hepatic focal lesions or areas of abnormal enhancement or metabolic activity.
- No biliary radicles dilatations could be noted.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta and IVC as well as pelvic organs
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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

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PAIDI FORUM

Name	Rahima - 200-15)			MRN ID	- Arif F. P. N. F.
Age	47 yr			Phone	89556-776
Menopausal status	Postmenstrual (Unknown)			Marital status	15VPO19P01
NRS SCORE				Family history	.15A11A9A01
BMI				REASON OF MDT	
CO morbidity					
Clinical Data					
BX:	ILC GII			R	Left V/A
ER:	8/8	PR:	6/8	HER2:	Geo
				KI67:	15%
Sonomammography	① Just LDD CX&M			△ △	
	② LITQ & USM			○ ○	
	③ Axial, Sagittal & Coronal			△ △	
Mets Workup:	PET/CT & FDG scan			TNM	
DVS low					
Recommendations:					
MDT OUTCOME & PLAN					
SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name: Doaa Farhy Hussein Ibrahim	Date : 22/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 3055/2024	Pat. ID: 78865

Combined bilateral breast mammography and complementary ultrasound

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- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Basma Mohamed

22/03/2025 2:39 PM



Preventive Program for Women's Health
Ministry of Health and Population

البرنامج الوقائي لصحة المرأة
وزارة الصحة والسكان



MDT form

As-patient ID:
1- Name: **فاطمة محمد الزامل**

Sex:

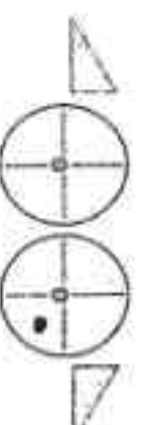
2- Medical number:
Female

8330862
DOB Year:

2025-2026

2-ID number: 37707001003141

Patient Age: 48 Y



Family history: **None** Phone: **01773519751** SHI: **SHI** Sex: **Female** Reason of MDT: **Asymptomatic lump**
Smoking status: **Non-smoker** MBS Status: **Not done** Group: **Group 1** Date of MDT: **2025-05-10** Date of next MDT: **2025-05-10**
Ca morbidity: **0** In request: **None** Pre-treatment: **None** Initial stage: **Not done** Live stage: **Not done**

Clinical data

Breast Imaging:
CT LUG BWS MAM
LT AXILARY IN BIRADS III
RT Breast BIRADS III

Mets Workup:
pet ct FDG avid
DVS lesion

ER	ER	PR	HER 2 Neu	SISH	Ki67
ILC	ER	6/8	0	SISH	15
ER (200-200x)	ER (200-200x)	PR (200-200x)	HER 2 Neu (200-200x)	SISH (200-200x)	Ki67 (200-200x)

T	N	M
CT3	3b	Solitary DVS
multicentric		

Sonomammography

PATIENT HISTORY

Initially invasive lobular carcinoma, rt breast BIRADS III
received a AC-->4 taxotere with marked regression
csmill breast BIRADS 6 residual area of non mass enhancement at the LoQ 2/4 octock

MDT OUTCOME & PLAN

FOR LEFT MAM THEN SBRT ON DS THEN HORMONAL TTT

SIGNATURE			
MEDICAL ONCOLOGY	PHYSICIAN SIGNATURE	RADIOLOGY	PATHOLOGY
SURGERY	H. Kalo	H. Kalo	H. Kalo
Psychiatric	H. Kalo	H. Kalo	H. Kalo